Before You Look at Any Place: Three Things to Get Clear About Your Wife's Care First

Three moves, in order, and what can wait until they're done.

The social worker would like to talk about where your wife goes next, and the names of places have started coming up. You know her day by heart. You'd like to know what to have ready for that conversation.

Start here, before you look at any place. Three things, in this order.

1 Ask her care team what they've confirmed

Ask the social worker, her nurse or her therapist what they have found she needs help with now, and ask them to say it in tasks instead of the name of a place. Write their answers beside what you've seen at home, and mark which is which. [1]

WHY THIS COMES FIRST Discharge planning is the process that works out the kind of care a patient needs after the hospital, and the planner will look to you for history and insights. Their findings and your account of her day belong on the same page.

2 List what the next arrangement has to be able to do

Make two lists. On the first, the help any arrangement would have to provide for her, written as things it must be able to do. On the second, what she'd like and what you'd like for her: the meals, the activities, being close to family. Anything about her health, her medications or how much watching over she needs goes on a third list marked “to be confirmed.” [2]

WHY THIS COMES FIRST The National Institute on Aging's first step in choosing a place is to consider the person's needs and wants. Keeping them on separate lists keeps both in view once the names of places start to run together.

3 Write down what's still open, and who answers it

Go down the “to be confirmed” list and turn each item into a question. Beside each one, write who it belongs to: her doctor, her nurse, her therapist, her pharmacist, the social worker or your care consultant. Then ask. The National Institute on Aging's advice when you're unsure is to ask a health care provider which services may be most helpful. [2]

WHY THIS COMES FIRST A question on paper stays a question until the right person answers it. In your head, it tends to get answered by a guess.

What can wait until these are done

- Calling or touring any place.
- Comparing one kind of setting with another.
- Working out which name fits her: assisted living, memory care or something else. That belongs to the people who assess her.
- Deciding. Your three lists are what the comparison starts from when it comes.

If the pace feels fast

The Family Caregiver Alliance's discharge guide says that if you're feeling hurried, it is reasonable to ask the discharge planning team to slow things down so nothing is overlooked. You can say that to the social worker directly.

What this guide doesn't do

It doesn't say what kind of care your wife needs or where she should get it, and it doesn't compare settings. It puts the days before that comparison in order.

Ready for the next step?

You've asked what her care team has confirmed, sorted the must-haves from the would-likes and written down what's still open. The care interview takes it from there: it goes through your wife's daily life area by area, including the parts that don't show up at home, and turns your answers into a Family Care Profile you can bring to any care conversation.

Create Your Free Family Care Profile ›

WHERE THIS COMES FROM

The description of discharge planning and the advice about asking to slow down follow the Family Caregiver Alliance. Thinking about needs and wants first, and asking a health care provider when unsure, follow the National Institute on Aging. The three moves and their order are the Family Care Roadmap's own.

1. Family Caregiver Alliance, Hospital Discharge Planning: A Guide for Families and Caregivers
2. National Institute on Aging, How To Choose a Nursing Home or Other Long-Term Care Facility

---

From the Family Care Roadmap · Richard Ladson, Certified Medicaid Planner. Draft for review, generated 2026-10-04.